



WHGMH - SARC PROCEDURAL GUIDELINE	
Conditions for emergency accommodation of SARC patients at KEMH	
Scope (Staff):	All SARC staff, Hospital Clinical Managers, Ward 6 staff, Gynaecology
Scope (Area):	SARC, Gynaecology
This document should be read in conjunction with the Disclaimer .	

Aim

To describe the conditions under which a female Sexual Assault Resource Centre (SARC) patient may be provided with emergency accommodation at King Edward Memorial Hospital (KEMH) and their subsequent management if admitted.

Keypoints

The Sexual Assault Resource Centre provides 24 hour a day, 7 days a week outpatient medical, clinical forensic medical and psycho-social care to patients of all genders aged 13 years and older who allege a recent sexual assault. SARC is housed in a stand-alone clinic with no in-patient beds and very limited nursing support. It is not staffed overnight. Very occasionally SARC patients may require emergency accommodation for social support and safety reasons. Ideally accommodation will be sourced externally, through existing community services. However in circumstances where this is not possible, overnight accommodation through KEMH may be considered. In the first instance, accommodation at Agnes Walsh House should be discussed with the Hospital Clinical Manager. All accommodation at Agnes Walsh House will be provided in accordance with the current clinical practice guideline “*Agnes Walsh House/Lodge: Transfer of a woman +/- her baby*”.

If overnight accommodation is not available, admission to Ward 6 may be considered but where Ward 6 has no available beds, admission to Emergency Centre (EC) is preferred.

Admission of female SARC patients for non-gynaecological medical concerns including suicidal ideation should not be to KEMH. (See Appendix 4: Exclusion criteria)

Patients alleging a recent sexual assault who are complaining of acute gynaecological issues, including heavy vaginal bleeding, will need assessment by a gynaecologist, (KEMH, FSH, Joondalup etc), ideally in combination with the SARC duty doctor if possible, and are not the subject of this procedure.

Risk

Non-compliance with this guideline will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	X
Breach National/State/Mandatory Policy	☐	Impact on Patient Safety	X
Breach professional standards	☐	Impact financial operations	☐
Damage NMHS reputation	X	Impact on Staff Safety	☐
Other	☐		

Compliance and Evaluation

Compliance can be evaluated by reviewing coding of the patient admission. Admission for reasons other than social support and safety would indicate that the policy is not being followed.

Roles and responsibilities

SARC team:

- To undertake a thorough SARC history and examination as per standard operating procedures.
- Ensure that all options to ensure patient social safety and security have been evaluated and eliminated before accommodation at KEMH is considered.
- Ensure that the patient has no serious medical concerns (bleeding, fractures, strangulation, severe mental health) that would warrant admission to a general hospital and not KEMH
- Follow the attached flow chart for the admission protocol
- Contact the Hospital Clinical Manager to discuss accommodation availability. Doctor to complete the Angus Walsh House Admission Letter (See Appendix 2).
- Refer directly to the Gynaecology Team of the Day, if no other accommodation services are available.
- Complete a detailed referral letter to the gynaecology team (see Appendix 3) including medications to be prescribed and attach a copy of SARC (yellow) Medical Care notes.
- The SARC doctor is the primary contact should any questions arise during the patient's stay in KEMH accommodation. Admitted patients will be managed by the Gynaecology Team of the Day.
- Review the patient the following day with respect to ongoing-care and discharge.
- Arrange follow-up SARC medical and psycho-social care as appropriate including liaison and referral with other agencies for patient care as necessary.

Hospital Clinical Manager:

- Provide information on availability and suitability of Agnes Walsh House accommodation, taking into account the current guidelines for admission.
- Assist with booking process.

Where a social admission is indicated:

Gynaecology/KEMH medical staff:

- Assume over-arching medical governance for the patient whilst admitted to KEMH (Ward 6/EC) including recognising and responding to clinical deterioration.
- Junior medical staff to complete appropriate documentation including medication charts as required.

Ward 6 nurses/KEMH EC midwifery staff:

- Provide standard regular nursing/midwifery ancillary care including standard 4 hourly observations.

Appendices

Appendix 1: Flow Chart for admission of SARC patients to KEMH

Appendix 2: KEMH Agnes Walsh House Accommodation Letter

Appendix 3: KEMH Admission Handover Letter

Appendix 4: Exclusion criteria for patients to be accommodated at KEMH

Definitions and abbreviations

Sexual assault: penetration of the mouth by a penis, penetration of the vagina and anus by a finger, penis or object where there is a lack of informed consent. (Ref: *Criminal Code Act Compilation Act 1913*, Chapter 31)

Related policies

KEMH Clinical Guidelines, Obstetrics & Gynaecology: *“Agnes Walsh House/Lodge: Transfer of a woman +/- her baby”*.

Useful resources (including related forms)

SARC : <https://www.kemh.health.wa.gov.au/our-services/service-directory/sarc>

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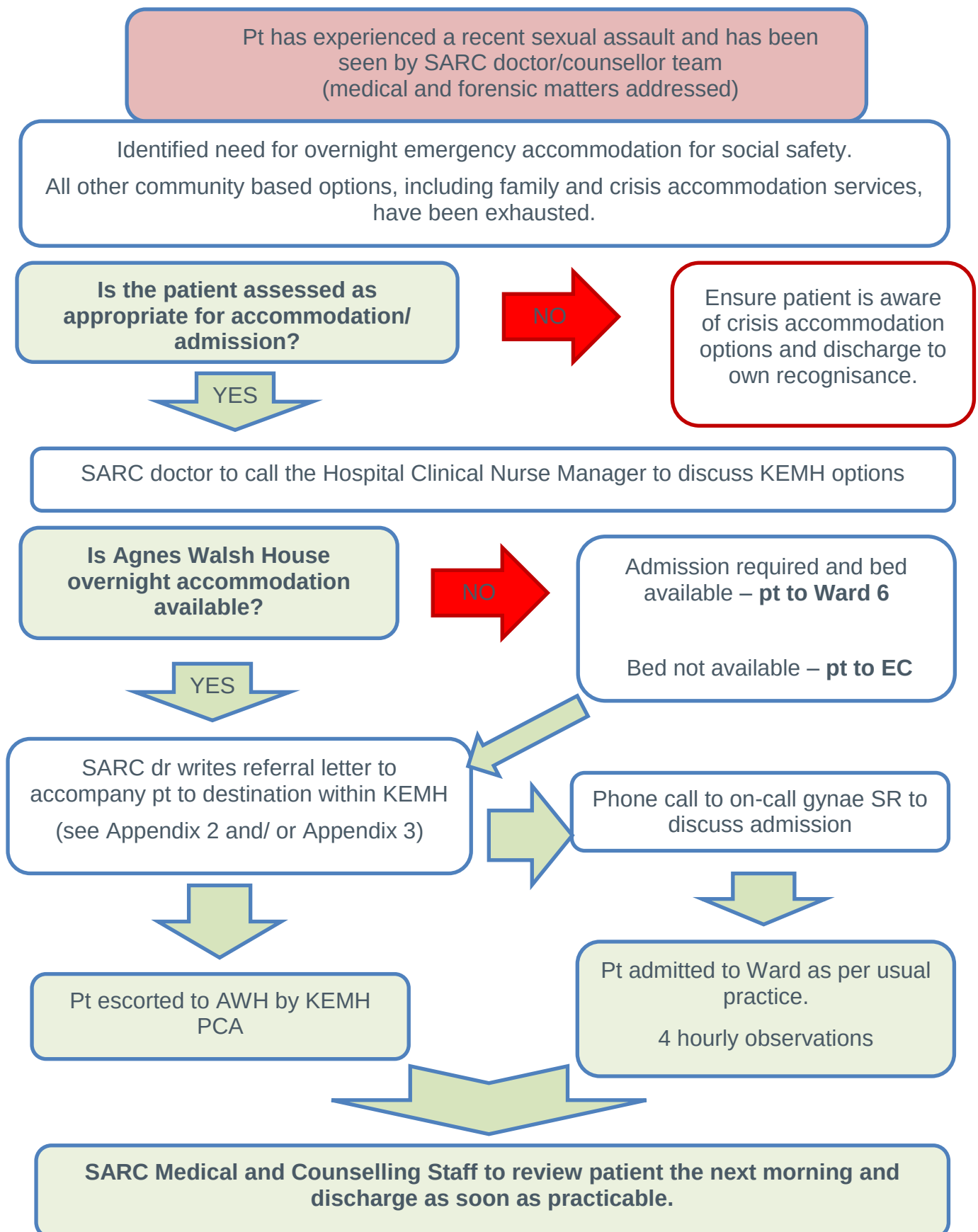
The health impact upon Aboriginal people has been considered, and where relevant incorporated and appropriately addressed in the development of this document (ISD No: 674).

This document can be made available in alternative formats on request for a person with a disability.

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Appendix 1: Flow chart for admission of SARC patients to KEMH¹



¹ If a male SARC patient requires emergency accommodation other alternative arrangements will be sought.

Appendix 2: KEMH Agnes Walsh House accommodation letter:

IDENTITY	Medical examination record Patient name: Date of birth: UMRN: Date: Dear HCM, Re: Patient name:
	The above person was seen at SARC today alleging a <i>type of</i> (eg penile-vaginal) sexual assault +/-physical assault. Police are/are not involved. <i>Pt's name</i> requires emergency accommodation at the KEMH Agnes Walsh house facility for social safety for the following reasons: • •
OBS	
BACKGROUND	No further history or examination is required and admission to hospital is not indicated. Please consider accommodation in the context of the current clinical practice guideline "Agnes Walsh House/Lodge: Transfer of a woman +/- her baby".
AGREED PLAN	I confirm that the SARC team will review tomorrow morning. Follow-up medical care will be arranged by SARC.
READBACK	Please contact the SARC duty doctor or myself at any time if you have any queries. Yours sincerely Dr _____ (signature) Name _____ Ph:6458 1828 or call KEMH switchboard and request the SARC doctor on-call

Appendix 3: KEMH Admission Handover Letter

IDENTITY	Medical examination record Patient name: Date of birth: UMRN:
	Date: Dear Dr, Re: Patient name:
SITUATION	The above person was seen at SARC today alleging a <i>type of</i> (eg penile-vaginal) sexual assault +/- physical assault. Police are/are not involved. I have undertaken a general physical and genito-anal examination and have noted the following: Please find attached SARC Medical Notes detailing history and examination. Further assessment should be undertaken at the discretion of the admitting Gynaecology Team. <i>Pt's name</i> requires an overnight admission to KEMH for social safety for the following reason: Emergency contraception has/has not been given. HIV prophylaxis is/is not required and has been dispensed with follow-up arranged.
OBS	Pt is hemodynamically stable: Pulse: , BP: , Resp rate:
BACKGROUND	PMH: Mental Health history: Medications: Allergies: Social history:
AGREED PLAN	The After Hours Hospital Clinical Manager has confirmed admission on <i>Ward 6 / EC</i>. I would be grateful if you prescribe the following: <i>eg Temezepam, paracetamol, ibuprofen or regular medication</i> I confirm that the SARC team will review tomorrow morning. Follow-up medical care will be arranged by SARC.
READBACK	Please contact the SARC duty doctor or myself at any time if you have any queries. Yours sincerely Dr _____ (signature) Name _____ Ph:6458 1828 or call KEMH switchboard and request the SARC doctor on-call

Appendix 4: Exclusion Criteria for patients to be accommodated at KEMH

Physical health

Physical injuries requiring further investigation and radiology/CT imaging

Physical injuries including head injury requiring regular neuro-observations

Non-fatal strangulation requiring on-going medical assessment and radiology

Mental Health

Suicidal ideation requiring mental health assessment

Risk of possible self-harm requiring regular observation and psychiatric review

Active expression of a plan to self-harm or commit suicide (but no plan)

Psychosis of any kind (schizophrenic or drug-induced) or thought disorder requiring psychiatric review

Social Safety

Homeless and expecting KEMH to arrange on-going long term accommodation or refuge accommodation

Alleged perpetrator at large and expressing a desire to commit homicide (pt then needs to be kept safe by the police)

NOT exclusion criteria:

Physical health:

Physical assault not requiring additional care beyond mild and regular analgesia

Non-fatal strangulation which has been assessed and cleared by an emergency physician

Mental health

Past/current history of mild/anxiety and/or depression which has not required admission to hospital

Emotional distress and crying (can be a normal response to trauma)

Expressing thoughts of low mood at SARC but with no intent, plan or mental health/depression history (mental health assessment undertaken at SARC)

Borderline personality disorder

Social safety

Patient lives alone